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Integrated Management of Childhood illness (IMCI)

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The State of Child Health

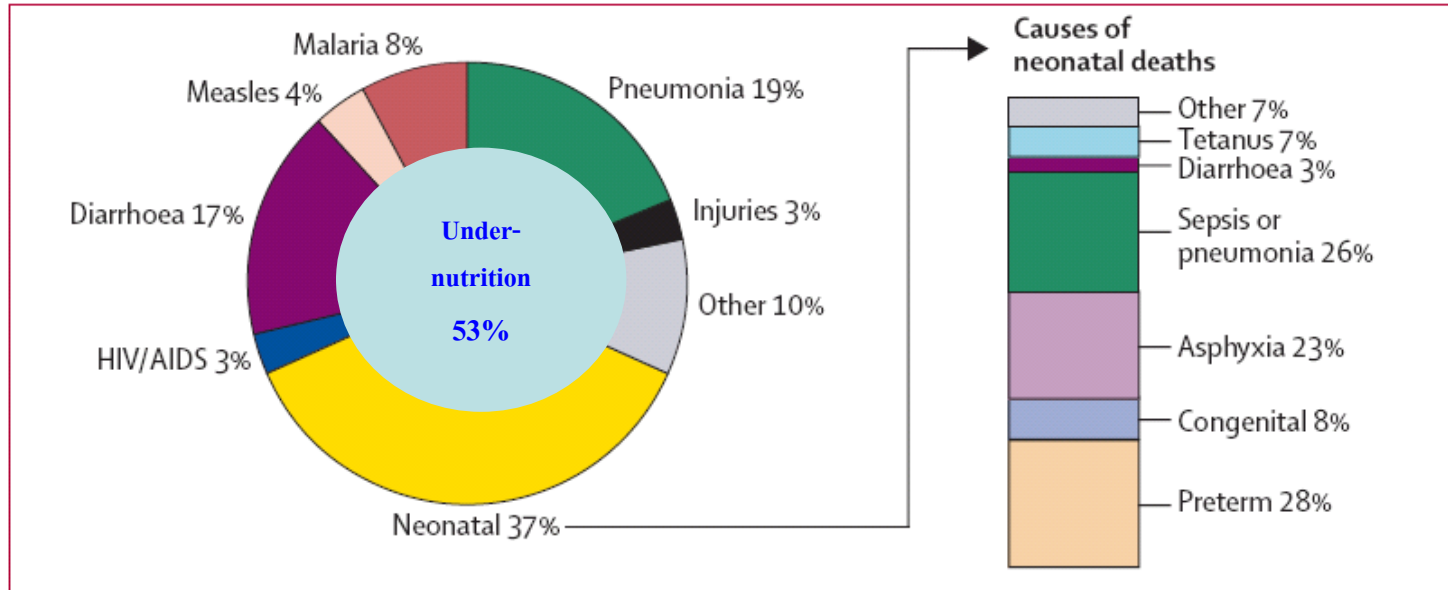
- 5.9 million children under age five died in 2015, nearly 16 000 every day
- 83% of deaths in children under age five are caused by infectious, neonatal or nutritional conditions.
- The large majority of these deaths are from preventable causes!



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Causes of Death in children

→ number one pneumonia
second diarrhea



Source: CHERG estimates of under-five deaths, 2000-03



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Background

Surveys of the management of sick children in most developing countries reveal that:

- Many children are not properly assessed and treated and that their parents are poorly advised.
- Diagnostic supports such as radiology and laboratory services are minimal or non-existent.
- Drugs and equipment are scarce.



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- Projections based on the 1996 analysis The global burden of disease indicate that common childhood illnesses will continue to be major contributors to child deaths through the year 2020.
- This assumption makes a strong case for introducing new strategies to significantly reduce child mortality and improve child health and development.
- WHO and UNICEF developed a strategy known as Integrated Management of Childhood Illness (IMCI).



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What is

IMCI?



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- IMCI is a strategy for reducing mortality and morbidity associated with major causes of childhood illness.
- The strategy includes preventive and curative interventions, which aim to improve practices both in the health facilities and at home
- It is an integrated approach to child health that focuses on the well being of the whole child *→ No sub specific parts*



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Interventions currently included in the IMCI strategy

Promotion of growth
(Preventive measures)

Response to sickness
(curative care)

Home

*Community/home-based interventions to improve Nutrition.

*Insecticide-impregnated Bed nets

*Early case management

*Appropriate care seeking

*Compliance with treatment

Health facility

*Vaccinations

*Complementary feeding

*Breastfeeding counseling

*Micronutrient supplementation

*Case management of:
ARI, diarrhoea, measles, malaria, malnutrition, other serious infection

*Iron treatment

* Antihelminthic treatment



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IMCI

- WHO and UNICEF used updated technical findings to describe management of these illnesses in a set of integrated guidelines for each illness.
- These guidelines have been adapted to each country



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Who can use IMCI?

- The IMCI process can be used by all doctors, nurses and other health professionals who see young infants and children less than five years old.
- It is a case management process for a first-level facility, such as a clinic, health center or an outpatient department of a hospital.



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Why is IMCI better than single-condition approaches?

- Children brought for medical treatment in the developing world are often suffering from more than one condition
- This overlap means that a single diagnosis may not be possible or appropriate and treatment may be complicated by the need to combine therapy for several conditions.



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Cont

- An integrated approach to managing sick children is, therefore, indicated as is the need for child health programs to go beyond single diseases and address the overall health of a child.

“Looking to The Child as a Whole”.



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Presenting complaint	Possible cause or associated condition
Cough and/or fast breathing	Pneumonia, Severe anemia P. Falciparum malaria
Lethargy or unconsciousness	Cerebral malaria, s. dehydration, meningitis, v. s. pneumonia. <i>for ex. caused by diarrhea</i>
Measles rash	Pneumonia, ear infection, diarrhea
In a very sick young infant	Sepsis, pneumonia



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Overall goal

The overall goal of IMCI is to:-

- 1) reduce the mortality and morbidity in under five children in relation to the major killers.
- 2) To promote improved growth and development of children.



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IMCI Components

Implementation of the IMCI strategy in countries involves the following three components

- 1) Improvement of health worker skills
- 2) Improvement of health systems
- 3) Improvement of family and community practices in relation to child health

These three components are complementary. They all need to be functioning well to fully benefit the child.



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Improves Health Worker Skills

- Case management guidelines *have schematic guidelines*
- Training of health providers (Doctors , Medical Assistants & Nurses) who look after sick infants and children up to 5 years (pre-service and in-service)
- Follow-up after training



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Improves Health Systems

- Targets first level health facilities
- Organization of work
- Availability of drugs and supplies
- Monitoring and supervision
- Referral pathways and systems
- Health information systems



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Improves Family and Community Practices

Exclusive Breastfeeding

- Complementary feeding
- Cont. feeding during illness.
- Using of iodized salt
- Routine vaccination
- Regular growth monitoring.
- Early care seeking.



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- Compliance to provider advice
- Home care of sick children
- Recognition of severe illness
- Proper waste disposal.
- Use of LLTN.
- Antenatal care
- TT for pregnant ladies.
- Proper nutrition for pregnant ladies.



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Benefits of IMCI

- Addresses major child health problems :

The strategy addresses the most important causes of childhood death and illness

- Promotes prevention as well as cure
- Improves health worker performance and their quality of care.



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Cont.

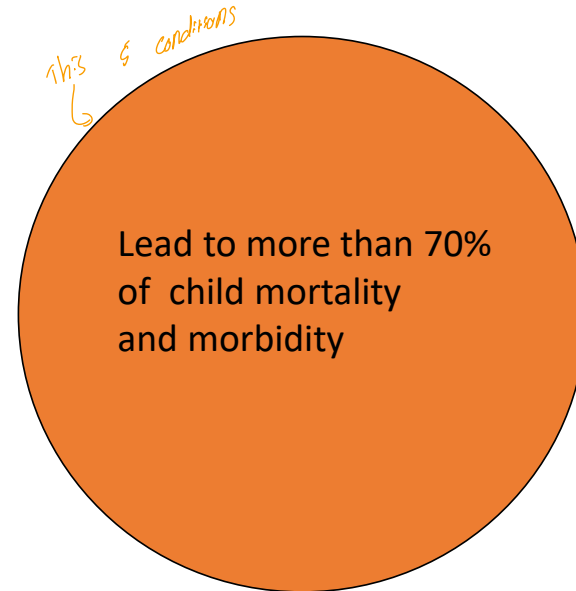
- IMCI improves health worker performance and their quality of care.
- IMCI can reduce under-five mortality and improve nutritional status, if implemented well;
- IMCI is worth the investment, as it costs up to six times less per child correctly managed than current care



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Diseases Covered By IMCI

- 1-Diarrhoeal diseases**
- 2-Acute respiratory infections**
- 3-Malaria**
- 4-Measles**
- 5-Malnutrition**





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Diseases NOT covered by IMCI

- The IMCI guidelines address the most important but NOT ALL of the major reasons a sick child or an infant is brought to the clinic with.



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Diseases NOT covered by IMCI

IMCI encourages the health provider to assess problems not included in IMCI charts. These are considered under the box :

ASSESS OTHER PROBLEMS



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Age Groups Covered By IMCI

- IMCI guidelines recommend case management procedures based on two age categories:-
 - Children age 2 months up to 5 years.
 - Young infants age up to 2 months



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1. Assessment of the child or young infant
2. Classification of the illness *→ whether child need to be treated { home, health center, emergency referral to hospital }*
3. Identification of the treatment
4. Provide treatment :
 - Pre-referral *→ like you give anti-biotic and send to hospital*
 - Medical treatment *→ specific treatment at health center*
 - Home management
5. counselling of the child's care giver (feeding problem , Mother Health)
6. follow -up care



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IMCI Case Management

Focused Assessment

Danger signs
Main Symptoms
Nutritional status
Immunization status
Other problems



Classification

Need to Refer

Specific treatment

**Home
management**



Counsel & Follow-up

Counsel caretakers
Follow-up



Treatment

Identify treatment

Treat



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Assess the Sick Young Infant, Age up to 2 Months



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Assess the Sick Young Infant , Age up to 2 Months

- AGE
- WEIGHT
- TEMPERATURE
- INFANT'S PROBLEMS → either SAs name of repeated problem
- INITIAL OR FOLLOW UP VISIT



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- Check for possible bacterial infection
- Check for the presence of Jaundice
- Check for diarrhoea
- Check for feeding problem or low weight
- Check for immunization
- Assess other problems *→ beside the 5 main condition*



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*Assess the Sick Child,
Age 2 months up to 5 years*



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Assess the Sick Child, Age 2 months up to 5 years

- AGE
- WEIGHT
- TEMPERATURE
- CHILD'S PROBLEMS
- INITIAL OR FOLLOW UP VISIT



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Check
General Danger Signs



CHECK
for
GENERAL DANGER SIGNS
in
ALL SICK Children



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Check for general danger signs for all sick children:

→ child with danger
sent to hospital

1- Unable to drink or breastfeed

2- Vomits every thing

3- Has the child had convulsions? → either prior or post

4- Unconscious, lethargic

5- Is the child convulsing now



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□ Assess major four symptoms:

1-Cough or difficult breathing

2-Diarrhoea

3-Fever

4-Ear problems



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- Check for nutrition, immunization, vitamin A supplementation and feeding problems.
- Assess other problems



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Classification the illness

The classification tables on the assess and classify have 3 ROWS .

COLOR of the row helps to IDENTIFY RAPIDLY whether the child has a SERIOUS DISEASE requiring URGENT ATTENTION.

Each row is colored either –

Referral to hospital
Red

– means the child has a severe classification and needs urgent attention and referral or admission for inpatient care.



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Classification the illness

- **YELLOW** – means the child needs a specific medical treatment such as an appropriate antibiotic, an oral anti-malarial or other treatment. *in health center and follow up*
- also teaches the mother how to give oral drugs or to treat local infections at home
-



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Classification the illness

only home treatment also may need follow up or reach danger signs for that condition

GREEN – not given a specific medical treatment such as antibiotics or other treatments.

The health worker teaches the mother how to care for her child at home.



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Cough or Difficulty of Breathing

SIGNS

CLASSIFY AS

IDENTIFY TREATMENT

- Any general danger sign or - Chest indrawing or Stridor in a calm child	SEVERE PNEUMONIA OR VERY SEVERE DISEASE	- Give first dose of an appropriate antibiotic } <i>PRR-referral treatment</i> - Refer URGENTLY to a hospital
Fast breathing	PNEUMONIA	- Give an appropriate oral antibiotic for 5days - Soothe the throat and relieve the cough with a safe remedy - Advise mother when to return immediately - Follow-up in 2days
No signs of pneumonia or very severe disease	NO PNEUMONIA, COUGH OR COLD	- If coughing >30days refer for assessment - Soothe the throat and relieve the cough with a safe remedy - Advise mother when to return immediately - Follow-up in 6days if not improving